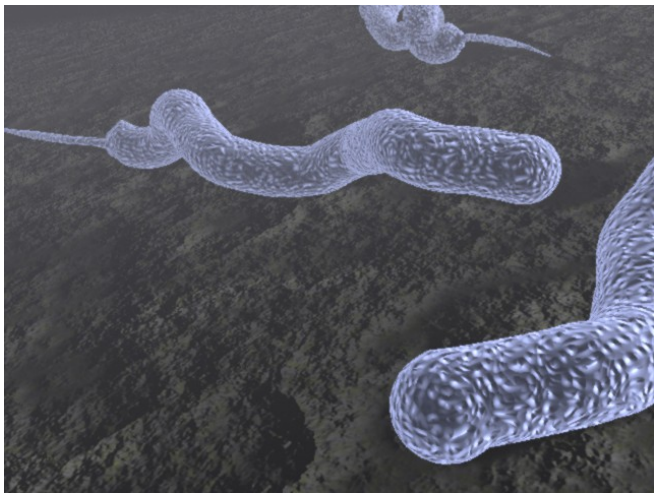
Guillain-Barre Syndrome *Prodigy Anesthesia*

Also known as acute idiopathic polyneuritis, Guillain-Barre syndrome (GBS) is a cell-mediated immune response against the axon of the peripheral nerve. It is often preceded by a mild course of gastrointestinal or influenza-like illness caused by the Campylobacter jejuni bacteria, Epstein-Barr virus, or cytomegalovirus. It has also been reported to be preceded by lymphomatous disease, vaccinations, and surgery.

The clinical presentation of GBS usually includes symmetrical paresthesias and weakness starting at the distal extremities and progressing to the proximal extremities and cranial muscles. Pain often accompanies the syndrome as well as sensory loss and arreflexia. The syndrome can be limited to only the legs or can progress to complete paralysis with autonomic dysfunction. SIADH and hyponatremia can also occur. It is diagnosed by the discovery of increased CSF protein despite a normal cell count. GBS is self-limiting and full recovery usually occurs.



Campylobacter jejuni

Succinylcholine should be avoided in these patients due to the potential for massive potassium release into the bloodstream. They also exhibit increased sensitivity to nondepolarizing muscle relaxants. General anesthesia is usually employed, but postoperative ventilatory support may be necessary. If autonomic dysfunction is present, then an arterial line is prudent. Regional anesthesia is controversial. It has been used successfully in obstetric patients, but some studies identify it as a possible trigger for GBS.

Fleisher LA. Anesthesia and Uncommon Diseases. 6th ed. Philadelphia, PA: Elsevier Saunders; 2012: 270-273.

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